

Sample hospital check-in transcripts

Synthetic, for testing consultation capture/extraction. Mix of routine ward-round check-ins and discharge-adjacent conversations, reflecting that not every doctor interaction carries the same weight.

Transcript 1 — Routine ward round check-in (low information density)

Setting: Day 3 of admission, pneumonia. Morning ward round.

Dr. Ahmed: Morning, Bill. How are you feeling today?

Bill: A bit better I think. Still coughing but not as bad.

Dr. Ahmed: Good, that's what we like to hear. Let's have a listen. [pause] Chest sounds clearer than yesterday. Any fevers overnight?

Bill: No, nurse said my temperature's been normal since last night.

Dr. Ahmed: Great. Bloods this morning look better too, your infection markers are coming down. We'll keep you on the same antibiotics for now and reassess tomorrow.

Bill: Okay. Am I close to going home?

Dr. Ahmed: Getting there. Let's see how today goes. I'll pop back tomorrow morning.

Bill: Sounds good, thank you.

Transcript 2 — Nurse-led check, no doctor present (should NOT be treated as a consultation)

Setting: Afternoon, same admission, day 3.

Nurse Priya: Hi Bill, just here to check your blood pressure and oxygen levels.

Bill: Go ahead.

Nurse Priya: [pause] Blood pressure's 128 over 82, that's good. Sats are 96%, also good. Any pain anywhere?

Bill: No, I'm alright.

Nurse Priya: Great, I'll note that down. Ring the bell if you need anything.

Bill: Will do, thanks.

Transcript 3 — Discharge-adjacent consultation (high information density, target case)

Setting: Day 5, cardiology ward, morning of planned discharge. Dr. Kelly (consultant) and Bill.

Dr. Kelly: Morning Bill, good news, we think you're ready to go home today.

Bill: Oh, that's great.

Dr. Kelly: So just to run through everything. You came in with what turned out to be a heart attack, we've treated that, and your heart's looking stable now on the scans. Going forward there's a few changes to your medications.

Bill: Okay.

Dr. Kelly: We're starting you on aspirin, one 75 milligram tablet once a day, that's to help stop clots forming — you'll take that every day going forward, indefinitely.

Bill: Every day, right.

Dr. Kelly: Right. We're also starting a new one, lisinopril, 10 milligrams once a day, that's for your blood pressure, also ongoing.

Bill: Lisinopril, got it.

Dr. Kelly: And your metformin for the diabetes stays the same, 500 milligrams twice a day, morning and evening.

Bill: Same as before, yep.

Dr. Kelly: One thing to flag — we're also thinking about starting a water tablet, a diuretic, just to help with some fluid we saw on your scan, but I want to double check with the team before we finalise that, so it might not be on your discharge paperwork yet. Worth asking your GP about that at your follow-up.

Bill: Okay, so that one's not decided yet.

Dr. Kelly: Not quite, yes. Now, for the next month, I'd avoid any strenuous activity, so no heavy lifting, no intense exercise, but gentle walking is good and encouraged. And try to eat a bit less salty food if you can, easier on the heart.

Bill: No heavy lifting, gentle walks, less salt. Got it.

Dr. Kelly: We'll get you a follow-up appointment with the cardiology team in about two weeks to check your blood pressure and your sugar levels, and we're also referring you to a cardiac rehab programme, they'll be in touch to book that in.

Bill: Two weeks, and someone will call about the rehab thing.

Dr. Kelly: Exactly. And just to remind you, you're allergic to penicillin, that's noted on your file, so make sure any new doctor or pharmacist knows that.

Bill: Yes, I always mention that.

Dr. Kelly: Perfect. Any questions for me before I go?

Bill: I don't think so, my daughter usually asks the questions and she's not here today.

Dr. Kelly: No problem, the discharge letter will have everything written down for her too. Take care, Bill.

Bill: Thank you, doctor.

Transcript 4 — Messy, realistic, interruptions and small talk (noise test)

Setting: Day 2, pneumonia admission. Morning ward round, phone rings partway through, family member briefly interrupts.

Dr. Ahmed: Morning Bill — oh, sorry, one sec. [phone rings, muffled] Yep, I'll call you back in five. Sorry about that, Bill, how are you doing?

Bill: Not too bad, actually my daughter's coming in a bit if that's alright.

Dr. Ahmed: Of course, no problem. So, right, let's see how you're doing. Chest's still a bit crackly on the right side but improving. How's the cough today, better or worse?

Bill: Bit better I'd say, still bringing stuff up though, sorry, excuse me — [coughs] — see what I mean.

Dr. Ahmed: Yep, that's expected at this stage, don't worry. We're going to switch you from the drip antibiotics to tablets today actually, since you're doing well enough — so that'll be co-amoxiclav, 625 milligrams, three times a day, and we'll also keep you on the clarithromycin, 500 milligrams twice a day, both for two more days.

Bill: Sorry, can you say that again, the amounts?

Dr. Ahmed: No worries — co-amoxiclav 625 milligrams three times a day, clarithromycin 500 milligrams twice a day, both for two days. It'll be written down for you anyway.

Bill: Ah okay, good, because I won't remember all that.

Dr. Ahmed: Don't worry, it's all in your discharge paperwork. Oh, and one more thing, we're holding off on your usual blood pressure tablet, the ramipril, for now, since your pressure's been a bit low. Your GP will need to decide when to restart that.

Bill: Right, so I'm not taking that one for now.

Dr. Ahmed: Correct, not until your GP says so. Alright, I'll let you get on, your daughter should be here soon. Take care.

Bill: Thanks doctor.

Transcript 5 — Heavy jargon, needs translation (complexity test)

Setting: Day 4, cardiology ward. Dr. Kelly discussing test results with Bill.

Dr. Kelly: Morning Bill. So your echo results are back. You've got mild LVEF reduction, ejection fraction's sitting around 45%, so borderline preserved, and there's some evidence of diastolic dysfunction as well.

Bill: I'm sorry, what does that mean exactly?

Dr. Kelly: Sorry, let me put that more simply — it means the pumping strength of your heart is slightly below normal, not severely, but enough that we want to keep an eye on it. Given that, plus the troponin rise we saw on admission and the ECG changes, we're confident this was an NSTEMI, a type of heart attack, rather than unstable angina.

Bill: Right, an NSTEMI.

Dr. Kelly: Yes. In terms of management, we're going to start you on an ACE inhibitor, that's the lisinopril I mentioned, plus continuing dual antiplatelet therapy with the aspirin. We're also going to keep monitoring your renal function given the ACE inhibitor, since it can affect kidney function in some patients, particularly alongside your background of type 2 diabetes and any pre-existing nephropathy risk.

Bill: So there's a chance this new tablet could affect my kidneys?

Dr. Kelly: It's something we monitor closely with a blood test, but it's usually well tolerated. We'll check your renal function again at your follow-up. Also, given the diastolic dysfunction, we're considering whether a diuretic might help with symptom control, but that's not confirmed yet, we'll finalise that with the wider team before discharge.

Bill: Okay, so that last one isn't settled.

Dr. Kelly: Correct, not yet. I know that's a lot of terminology, is there anything you'd like me to go over again?

Bill: Maybe just the bit about my heart's pumping again, and I'll ask my daughter to look at the rest with me later.

Dr. Kelly: Of course — basically, your heart's pumping slightly less strongly than we'd like to see, that's why we're starting these medications, to protect it and help it work more efficiently over time. We'll keep monitoring it closely.

Bill: Okay, that makes more sense, thank you.

Why these five are structured this way

Transcript 1 — Plenty of talk, almost no actionable content (no medication changes, no new instructions). Good test case for whether extraction correctly outputs "nothing new to report" rather than manufacturing content.

Transcript 2 — No doctor present at all, routine vitals only. Good test case for whether the system correctly does not flag this as a consultation worth extracting from.

Transcript 3 — The target case: dense, multiple medication changes with status (new/continued/undecided), an explicit "not finalized yet" item (the diuretic) that should surface as a flag once cross-checked against the discharge note, lifestyle instructions, follow-up plan, and an allergy reminder.

Transcript 4 — Real-world noise: interruptions, a phone call, coughing, small talk, a repeated question. Tests whether extraction can find the signal (co-amoxiclav 625mg TDS, clarithromycin 500mg BD, both 2 days, ramipril held pending GP review) inside a messy, non-linear conversation.

Transcript 5 — Heavy clinical jargon (LVEF, diastolic dysfunction, NSTEMI, dual antiplatelet therapy, nephropathy) that the doctor eventually simplifies mid-conversation. Tests both translation quality and repeated detection of the unconfirmed diuretic flag.